The Hillingdon Hospitals
NHS Foundation Trust

Maternity Accident and Emergency (A&E) Pathway during pregnancy and up to 28 Days Postpartum

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Version	Author	Summary of changes
2.0	Gillian Pearce CNST midwife Sarah- Jane Lam, Consultant Obstetrician	Guideline reformatted
3.0	Sarah- Jane Lam, Consultant Obstetrician Taruna Agrawal, Clinical Fellow for Risk and Maternal Medicine	Title changed, full review

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1. Aim/Purpose

1. The purpose of the guideline is to ensure optimum care for all pregnant women 18 weeks to 28 days postpartum seen in the emergency department or elsewhere in the hospital.
2. To outline the system for ensuring that the obstetric team and on call obstetric consultant **are aware of all sick pregnant women** in the hospital who have an obstetric or non-obstetric problem.
3. To outline referral pathways and communication between various healthcare professionals to ensure high quality management for the pregnant woman.

2. Introduction

The Triennial confidential enquiries into maternal deaths (MBRRACE) have highlighted the need for involvement of obstetricians in the emergency care and ongoing management of women admitted into hospital.

The care of pregnant women requiring hospital attendance and admission must be discussed and planned with the local obstetric team.

Definitions

The Midwifery Led Pathway is for all healthy women with uncomplicated pregnancies. The midwife is the lead professional and first point of contact, to confirm, book, and plan care.

The Maternity Team Pathway is for women with special medical/social or obstetric needs that require a consultant obstetrician as the lead professional. Care can also be shared with the team midwives, GP's and other professionals as appropriate.

3. The Guidance

3.1 Reasons for referral to obstetrics and gynaecology (O&G) team or Duty Midwifery manager from outside maternity unit

Presenting Problem:	Obstetric & Gynaecology staff member to see patient
Medical Problems e.g. Inflammatory bowel disease, cardiac disease, haematological disorders, severe asthma, diabetic ketoacidosis, suspected thrombosis (deep vein thrombosis/ pulmonary embolism)	Experienced specialty registrar
Mental health problems including overdose (specialist midwifery staff should be informed as a minimum)	Midwifery and/or Experienced specialty registrar
Domestic Violence (Safeguarding midwifery staff should be informed as a minimum)	Midwifery or Experienced specialty registrar
Surgical problems: Abdominal or pelvic pain (staff should be aware of the guideline for ectopic pregnancy) – discuss with O&G.	Experienced specialty registrar

Vaginal bleeding- discuss with O&G team or arrange EPAU follow up if light bleeding. If bleeding is heavy refer to Gynaecology specialty doctor on bleep 5460 or 5874 out of hours.	Experienced specialty registrar
Post-operative/procedure readmission	Experienced specialty registrar
Other: **Pyrexia or any evidence of infection** – to include post miscarriage/delivery **Abnormal blood results**: including raised CRP **Vomiting or diarrhoea** **Trauma and any pregnant patient in whom surgery is considered**	Please discuss with specialty registrar

Alert Maternity Bleep Holder or Maternity Triage of any attendance of a pregnant woman over 18 weeks up to 28 days postpartum in A&E whether requiring admission or not.

(Consider child protection and vulnerability, teenagers, domestic violence including women with psychiatric history).

Any serious antepartum or postpartum haemorrhage must be admitted directly to labour ward.

3.2. Process of referral to obstetrics and gynaecology for patients with obstetric problems

3.2.1 Early pregnancy (<18 weeks) and Gynaecological problems

These women **should be seen in A&E or EGAU as appropriate and after discussion with EGAU Specialty nurse.**

Please see gynaecology guideline on referral pathways for pregnant patients and gynaecological patients attending as an emergency See Appendix 1 and 2 for gynaecological pathways.

3.2.2 Women >18 weeks pregnant to 28 days postpartum with pregnancy related problems and STABLE should be seen in maternity

These women are triaged and seen on Labour ward, Antenatal ward or day assessment unit as appropriate.

If the woman attends A&E instead of maternity, discuss with maternity duty manager (Bleep 5763) as to appropriate area in maternity to be seen (Refer to the flowchart in Appendix 3 'Maternity A&E Pathway >18/40 to 28/7 Postpartum').

3.2.3 Women with pregnancy related or postpartum problem presenting to A/E whose condition is unstable

The woman should be moved to **A&E Resuscitation area**, attended to immediately by the A&E doctor and the O&G specialty registrar (on bleep 5890 and 5884) emergency bleeped. All interventions and observations documented.

3.3. Pathway of care of pregnant women with a non-obstetric problem requiring admission

Women who are pregnant but who present with non-pregnancy related problems requiring admission must all be **discussed with the on-call obstetrics and gynaecology (O&G) team** and reviewed as appropriate; even if they are referred and primarily seen by other specialties. O&G team to document details on labour ward handover board and sheet.

In A&E, if in any doubt whether the problem is pregnancy related or not, the A&E doctor or shift leader will discuss with the O&G specialty doctor on call while the woman stays in A&E. Discussion will also take place regarding the most appropriate area/specialty/ward where the woman will be admitted.

3.4. Communication between teams

Communication and multidisciplinary teamworking is essential to good clinical care. The respective teams must liaise during the woman's admission. See Appendix 4 for an explanation of the roles and responsibilities for staff in maternity.

All communication must be documented on Cerner.

If admitted:

- The admitting team must contact the obstetric specialty doctor to arrange initial review and further management as appropriate.
- All outliers (women on hospital wards outside maternity unit) will be written on the labour ward whiteboard, the handover sheet and in the maternity outliers book
- The outliers will be discussed at each maternity handover.
- The labour ward clerk will call the clinical site practitioner (CSP) daily to determine if any pregnant patients have been admitted in the last 24 hours and record this in the outlier book.
- The obstetric team will liaise with the admitting team daily.

3.5 Communication with the consultant obstetrician

The on-call O&G consultant will be made aware of all sick pregnant women in the hospital whether they have an obstetric or non-obstetric problem

3.5.1. Process of communication with consultant

The O&G consultant must be informed at handover of all admissions by specialty obstetric doctors.

Out of hours all sick pregnant patients must be discussed with the consultant on-call by the obstetric specialty doctor, duty midwifery manager or other specialty consultant.

A plan for review and management must be made and documented in the patient record and the handover sheet.

Communication at a senior level is paramount to good clinical care and in sick patients will occur between consultants of different specialities.

3.6 Management

- The aim is to ensure the woman's care is discussed and planned with the local obstetric team by the admitting team
- The obstetric team will contact consultant obstetrician on call as appropriate.
- The obstetric team can help with assessment of fetal well-being, investigations and treatment.
- **A clear management plan must be documented for ALL patients**
- Radiological investigations are not contraindicated during pregnancy where there is a significant clinical indication. Discuss with obstetric team.

Pregnant women <18 weeks pregnant with pregnancy related problems are usually admitted to gynaecology ward or seen in Emergency Gynaecology Assessment Unit (EGAU)

See gynaecology guideline on referral pathways for pregnant patients and gynaecological patients attending as an emergency

If pregnant women are admitted elsewhere in the hospital, the CSP will inform the local obstetric team and ward nurses must call the O&G specialty doctor for review as required, communicating the clinical details.

1. The PAR (Patient at Risk) scoring charts should be used.
2. They will be added to the gynaecology ward round list by the gynaecology specialty doctor.
3. They will be reviewed daily on the gynaecology ward round

Pregnant women <18 weeks pregnant with non- pregnancy related problems are usually admitted to a general ward

1. The admitting team and/or CSP will inform the local obstetric team, and the admitting team and ward nurses should call the O&G specialty doctors for review as required communicating the clinical details.
2. The PAR scoring charts will be used.
3. They will be added to the gynaecology ward round by the Gynaecology specialty doctor.
4. They will be reviewed by the gynaecology team daily.

Pregnant women >18 weeks pregnant up to 28 days postpartum with pregnancy related problems should be admitted on the Maternity wards – labour ward, antenatal or postnatal as appropriate.

1. MEWS charts must be used for these women.
2. Any concerns regarding inpatients must be communicated to the labour ward coordinator or O&G specialty doctor as appropriate depending on the clinical situation. .
3. They will be reviewed daily by a senior member of the obstetric team

Pregnant women >18 weeks pregnant up to 28 days postpartum with non-pregnancy related problems should be admitted on the maternity wards (labour ward, antenatal or postnatal) or general wards depending on their diagnosis

1. MEWS charts must be used for these women if admitted to maternity.

2. Any concerns regarding inpatients must be communicated to the Labour ward coordinator or O&G specialty doctor as appropriate depending on the clinical situation.

3. The clinical details and need for review must be communicated and documented.

4. The surgical or medical team to call the obstetric team if any concerns

5. They will be reviewed by the obstetric team daily.

3.7 Follow up of pregnant women discharged from non-maternity departments

- If these women are seen by A&E doctors, referred to obstetric team or discharged from A&E with follow up, all communication must be recorded and arrangements for follow up as required documented.
- They will be informed to represent if the problem persists
- The obstetric team must consider a referral to the maternal medicine or relevant specialist obstetric/ medical clinic if appropriate.
- If the woman represents with a recurrent problem the obstetric consultant must be informed and a management plan with a more detailed review made.

4.0. Associated Guidelines, Polices and Documents

- Referral pathways for pregnant patients and gynaecological patients attending as emergency
- The early pregnancy assessment unit guidelines
- A&E EGAU appointments booking form
- The early recognition of the severely ill pregnant woman guideline
- Maternity triage guideline
- Clinical risk assessment (Labour) guideline
- Roles and responsibilities of maternity bleep (5763) holder guideline
- Labour Ward Staffing
- Handover of care (on site) guideline

5.0 Training Need Identified

There is no specific training course associated with this guideline.

6.0 Implementation and Monitoring

All guidelines, protocols and procedures are available on the intranet. All staff have a responsibility to check the Trust's intranet and ensure that they are referring to the most recent issue of the document. The weekly general information bulletin includes notification of publication of new/updated documents.

7.0 References

1. CNST Maternity Clinical Risk Management Standards version 1; 2012/13
2. An Organisation-wide Document for the Care of Pregnant Women Seen in the Emergency Department or Elsewhere in the Hospital. Version 2, March 2012.
3. Confidential Enquiry into Maternity and Child Health (2004) Why Mothers Die 2000-2002 London: RCOG Press
4. Confidential Enquiry into Maternity and Child Health (2007) Saving Mothers' Lives: Reviewing maternal deaths to make motherhood safer – 2003-2005 London: CEMACH
5. Department of Health (2004) Maternity Standard, National Service Framework for Children, Young People and Maternity Services London: COI
6. CMACE. Saving mothers' lives; Reviewing maternal deaths to make motherhood safer (2006 – 2008) 8th report into the Confidential Enquiries into maternal Deaths in the United Kingdom. March 2011 RCOG.
7. Royal College of Anaesthetists, Royal College of Midwives, royal college of Obstetricians and Gynaecologists, Royal College of Paediatrics and Child Health, (2007). Safer Childbirth: Minimum Standards for the Organisation and Delivery of Care in Labour. London; RCOG Press. Available at www.rcog.org.uk

Appendices

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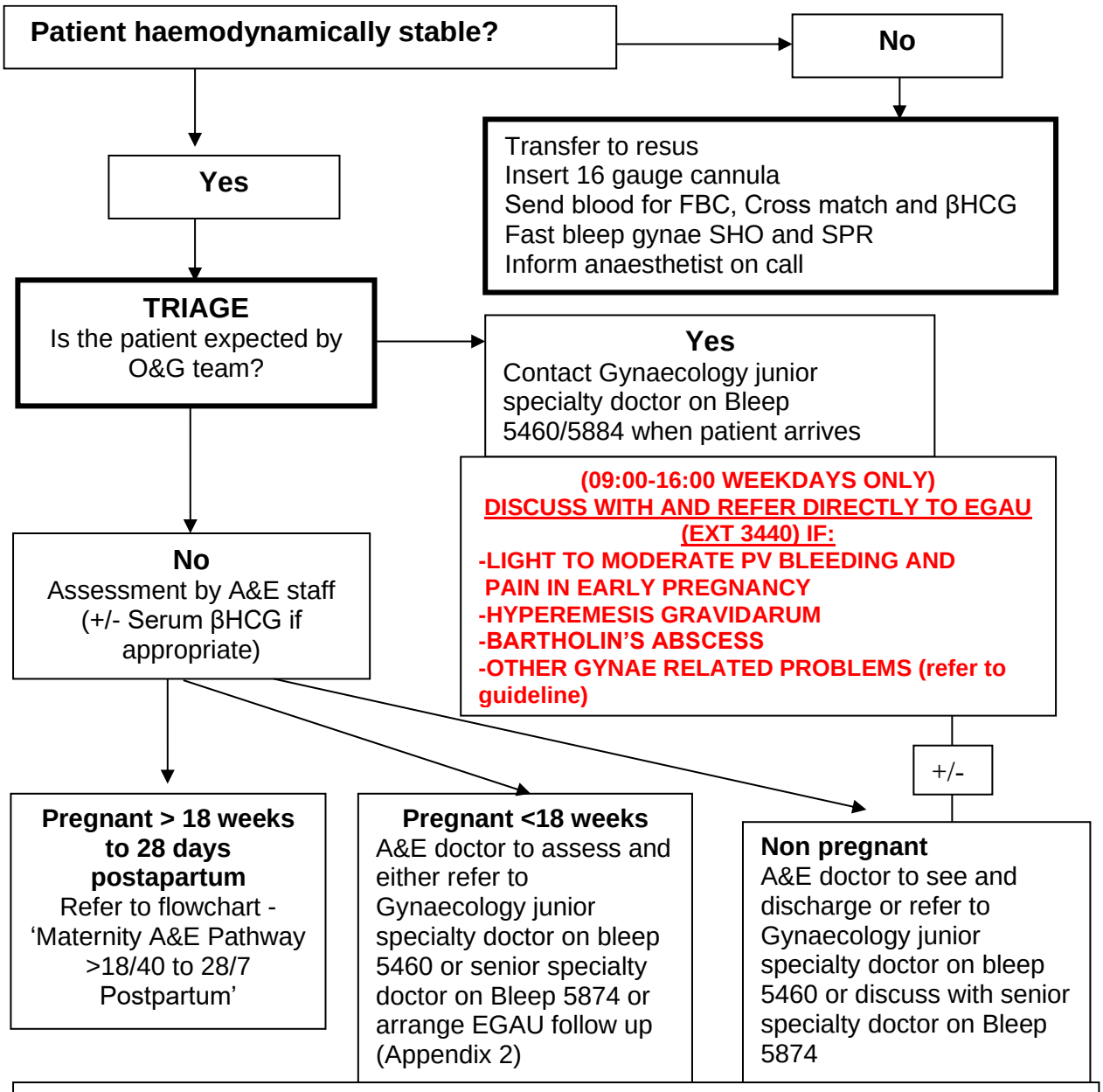
APPENDIX 1 - PROCEDURE FOR REFERRAL FOR PREGNANT PATIENTS AND GYNAECOLOGY PATIENTS ATTENDING A&E



Obstetric and Gynaecology specialty doctor on call – Bleep 5874

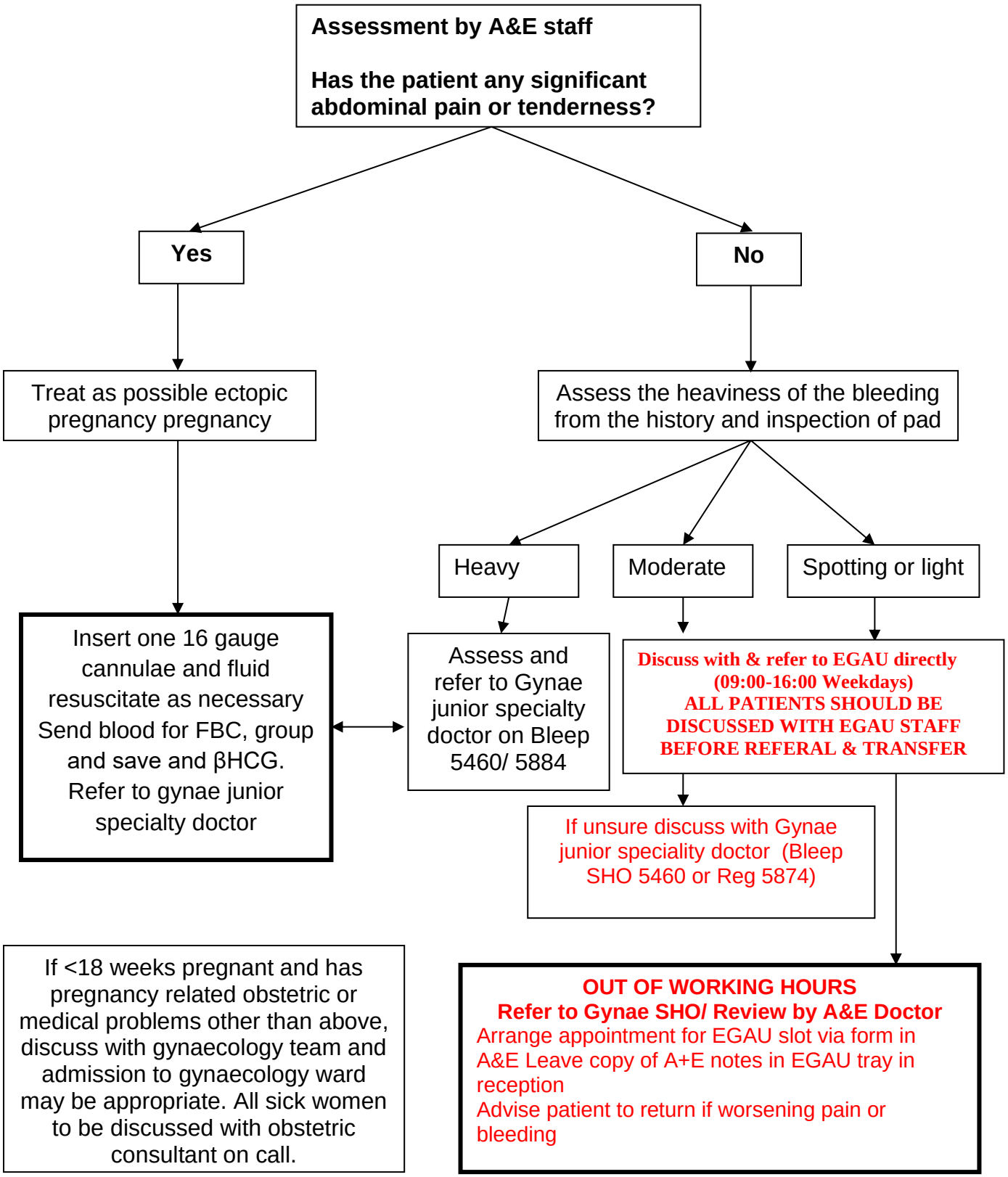
Gynaecology junior specialty doctor – Bleep 5460; Alternatively Bleep 5884 between 17:00 and 8:00am

Obstetric junior specialty doctor– Bleep 5884

If a pregnant woman with a non- obstetric problem is seen in A&E and admitted outside the maternity service you must contact the Maternity Bleep holder on 5763 +/- obstetric specialty doctor (Bleep 5890) who will contact the obstetric consultant as appropriate

No patient is to be sent to EGAU without prior discussion with EGAU Staff

APPENDIX 2 - ASSESSMENT OF THE STABLE PREGNANT PATIENT (<18 WEEKS) WITH VAGINAL BLEEDING OR PAIN IN A&E



Appendix 3 Maternity A&E Pathway >18/40 to 28/7 Post Partum

Bleep Numbers
MBH (maternity bleep holder) - **5763**
Obs/ Gynae Reg – **5890/5874**
Obs / Gynae SHO - **5884**
A/E Reg - **5737**
A/E Shift Leader - **5836**
CSP - **5838**
Labour Ward Ext – **3463**

Woman presents to A&E reception

Reception to bleep **5836**

A&E Bleep holder takes history

Alert Maternity Bleep Holder of any attendance of a pregnant woman in A&E
(Consider child protection and vulnerability, teenagers, domestic violence including women with psychiatric history)
Any serious antepartum or postpartum haemorrhage must be admitted directly to labour ward

If pregnancy related or post partum problem and woman stable

Alert Maternity Bleep Holder / Maternity Triage

MBH to discuss case with Obs speciality doctor if necessary, follow instructions

Agree plan and send to Maternity triage or appropriate area

If pregnancy related or post partum problem and woman **NOT** stable

Move woman to Resus
Fast bleep **2222**
Obstetric Team
5890 5884 5737

Document all interventions

If problem not pregnancy related (Discuss with Obs Reg if any doubt), woman to stay in A&E

Book woman into A&E Fast track to ATC& Triage within 15 minutes
Notify 5836 & 5737

CSP to inform maternity team daily of all women admitted within the trust including A&E Obs ward (Obstetric team to record admissions on handover sheet on LW)

Be alert for maternity sepsis & use Trust guidelines as directed

Activate 2222 to fast bleep Obstetric team
08:00-20:00 Obs Consultant on call present on site

A&E doctors document on A&E Card Obstetric doctor contacted

APPENDIX 4 – Explanation of roles and responsibilities

Maternity Triage is a midwifery- led assessment area located on 1st floor Maternity. Triage provides a 24hr service, 7 days a week and sees women with pregnancy related problems from 18 weeks onwards.

Labour ward coordinator is senior midwife on labour ward with the remit of managing the shift efficiently and effectively (Refer to guideline – 'The role of a labour ward coordinator)

Experienced doctor in the obstetrics & gynaecology team would be the senior obstetric specialty doctor or obstetric consultant. The obstetric specialty doctor must be aware of all admissions under the obstetrics & gynaecology team and review these women as appropriate. All sick women and complex cases should be discussed with the O&G consultant on call.

A&E Bleep holder (A&E shift leader on Bleep 5836) will take initial history for all pregnant women attending A&E and triage and follow the referral pathways outlined below.

A&E Doctors - A&E Registrar (Bleep 5737) or SHO may see pregnant women initially and refer on to obstetric team as per referral pathways below (**Appendix 1 & 2**).

Clinical Site Practitioner (CSP on bleep 5838) will inform MBH daily of all pregnant women admitted within the trust outside maternity including A&E Obs ward.

On call obstetric consultant or O&G consultant

The role of the on call obstetric consultant will include as a minimum:

- The on call O&G Consultants must be available for discussion and help as requested by the O&G specialty doctor or labour ward coordinator or other professionals.
- The consultant is present on site between 8:00 – 20:00 on weekdays and available as called from home usually within half an hour for emergencies (refer to the guideline 'The Role of the obstetric consultant on the labour ward at Hillingdon Hospital'
- The on call consultant must be involved in the care of all sick pregnant women and lease with consultants from other specialities as appropriate
- Recording of all communication.

Local obstetric Team – O&G specialty doctor

The role of the local obstetric team will include as a minimum:

- Will respond promptly to referrals from A&E staff. They may give advice, review the patient or arrange appropriate follow up as required.
- If the O&G specialty doctor is unsure about management, this must be discussed with the on call O&G consultant.

- All sick pregnant patients must be discussed with the on call O&G consultant, communicating the clinical details and need for review. They must be discussed with the O&G consultant before referring onto other specialties as appropriate.

- Recording of all communication.

Labour ward coordinator
The role of the labour ward coordinator will include as a minimum

- Communication with the on call O&G specialty doctor as required, communicating clinical details and indicating the need for review. In case of emergency situations – escalation to on call O&G consultant as appropriate.

- The arrangement/provision of appropriate midwifery care (Refer to guideline -– 'The Role of a labour ward coordinator)

- Recording of all communication.